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The AI Prompt Pack
for Indian Doctors & Clinics

85 ready-to-use prompts for patient communication, clinic marketing, online reputation, OPD admin and practice growth - written for Indian medical practice, NMC-aware and DPDP-aware.

2026-27 EDITION

For practising doctors, clinic owners, hospital marketing teams and medical practice managers across India.

What is inside this pack

Every prompt in this pack is a template you copy, fill in the square brackets, and paste into any AI assistant. The sections run in the order a busy clinic actually uses them - talking to patients first, then getting found, then staying compliant.

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Read the two sections before Section 1

Start Here (page 3) tells you the three privacy rules, which AI tool to use, and the five-part prompt formula that makes every prompt in this pack work better.

The Compliance Primer (page 5) is the part that protects your registration. It explains what an AI assistant may never do in a clinic, and how the DPDP Act 2023 and the NMC ethics regulations apply to what you type.

Start Here

This is not a book you read once. It is a working folder of 85 prompts, sorted by the job in front of you. When a patient sends a worried WhatsApp message at 9 p.m., you open Section 1. When your Google rating needs a reply, you open Section 5. When you must write a referral letter, you open Section 8.

Why this pack exists

India has 13,88,185 registered allopathic doctors and 7,51,768 registered AYUSH practitioners, a doctor-to-population ratio of about 1:811 that is better than the World Health Organization's 1:1000 benchmark (Press Information Bureau, February 2026). The constraint is no longer the number of doctors. It is the paperwork, the phone, the inbox and the marketing that sit between a doctor and the next patient.

At the same time, the patient's first step has moved online. In Indian metros, roughly 68 per cent of first-time patient bookings begin with a Google Maps search - a search like "cardiologist near me" or "dental clinic in Indiranagar" (Ichelon Consulting, 2026). A study of 245 leading Indian healthcare brands found the average Google rating was 4.78 stars, with about 22.9 new reviews arriving every month - and only 60.5 per cent of those reviews ever getting a reply.

An AI assistant will not diagnose your patients and should never try. But it is very good at the layer around the consultation: the plain-language explanation, the reminder message, the review reply, the discharge sheet, the first draft of a blog post. That is exactly what this pack covers.

The three rules before you paste anything

1. Never paste identifiable patient data. No names, no phone numbers, no ABHA numbers, no addresses, no dates of birth, no photos. Strip them first. A prompt that needs patient context should use a placeholder such as [45-year-old man, type 2 diabetes, controlled] - the clinical shape, not the person.
2. The doctor, not the AI, makes every clinical decision. An AI assistant can draft, rephrase and summarise. It cannot diagnose, prescribe or counsel a patient directly - the NMC Telemedicine Practice Guidelines, 2020 are explicit on this point.
3. Everything an AI writes is a first draft. You read it, you correct it, you own it. A message sent under your clinic's name is your professional statement, no matter who typed the first version.

Which AI tool should you use

Any of the mainstream assistants will run every prompt in this pack: ChatGPT, Google Gemini, Anthropic's Claude and Microsoft Copilot all have free tiers that are more than enough for clinic work. Pick one and stay with it for a month so you learn its habits. Two practical notes: never paste patient identifiers into any of them, and check your clinic's or hospital's own IT policy before you do - many hospital chains now have an approved list.

The five-part prompt formula

Every prompt in this pack is already built on this formula, but you will get better results once you can see it. When you adapt a prompt, keep all five parts:

Part	What it does	Example
Role	Tells the AI who it is writing as	"You are the front-desk manager of a small dental clinic in Pune."
Task	One clear instruction	"Draft a WhatsApp reply to a patient worried about a toothache."
Context	The facts, without identifiers	"Pain for two days, no swelling, no fever, clinic opens at 10 a.m."
Format	Length and shape of the answer	"Under 90 words, warm, no medical jargon, end with one question."
Guardrail	The line not to cross	"Do not diagnose, do not name any medicine, do not promise a cure."

Drop the guardrail and you get confident nonsense. Keep it and you get a usable first draft in seconds. That single habit is worth more than any other advice in this pack.

How the sections are organised

Section	What it covers	Prompts
1. Patient Communication	WhatsApp, phone and in-clinic messages	12
2. Patient Education	Plain-language explainers and care sheets	8
3. Appointments & Follow-up	Confirmations, reminders, no-show recovery	8
4. Google Business Profile	Getting found by "near me" searches	8
5. Reviews & Reputation	Asking for and replying to reviews, safely	8
6. Clinic Content & Social	Instagram, reels, blogs, awareness days	8
7. OPD Admin & Documentation	Notes, summaries, certificates, records	8
8. Letters & Professional Writing	Referrals, case reports, bios	7
9. Practice Growth & Retention	Recall, feedback, new services	7
10. Compliance & AI Safety	Consent notices, AI-use policy, safe wording	6
11. Regional Languages	Hindi and other-language patient material	5

What this pack is not

It is not a clinical decision-support tool, and it does not replace the NMC Telemedicine Practice Guidelines, the NMC Professional Conduct regulations or your own professional judgement. It is a writing assistant's playbook. Treat every output the way you would treat a draft from a junior colleague: useful, fast, and needing your signature before it goes out.

A note on the numbers in this pack

Figures from the Press Information Bureau, the NMC, the National Health Authority, the DPDP Act 2023 and published 2026 healthcare-marketing research are quoted with their source in the text. Where a number is an estimate from a survey rather than an official count, the text says so. Nothing here is invented.

The Compliance Primer

This is the most important section in the pack. India's medical regulators and its data-protection law both apply to the words your clinic puts out - including the ones an AI drafted. Read this once properly, then keep it as the reference you return to.

The four things an AI assistant must never do in your clinic

1. Diagnose, prescribe or counsel a patient directly. The NMC Telemedicine Practice Guidelines, 2020 (prepared with NITI Aayog) state plainly that technology platforms based on AI or machine learning are not allowed to counsel patients or prescribe medicines. Only a registered medical practitioner may do that, and the final prescription or counselling must come directly from the doctor. An AI can help you draft; it cannot be the voice the patient acts on.
2. Solicit patients. The Indian Medical Council (Professional Conduct, Etiquette and Ethics) Regulations, 2002, Regulation 6.1, make soliciting patients - directly or indirectly - unethical. A doctor may not let their name be used in advertising that invites attention to their skill, qualifications or achievements. Marketing copy for a clinic must inform, not solicit. Section 10 gives you the wording test.
3. Publish anything that identifies a patient. Regulation 7.14 of the same code bars a doctor from disclosing a patient's secrets, and a public review reply is no exception. The DPDP Act 2023 adds a second layer: personal data may be processed only for a lawful purpose such as the person's own consent.
4. Confirm a patient relationship in public. This is the single most common mistake. Replying to a review with "we are sorry your surgery at our hospital did not go well" publicly confirms that the reviewer was a patient. That is a data disclosure. Section 5 gives you reply templates that avoid it.

The DPDP Act 2023, in plain words for a clinic

The Digital Personal Data Protection Act, 2023 (with the DPDP Rules, 2025) governs how your clinic handles a patient's digital personal data - and health data is among the most sensitive categories. The parts that touch your day-to-day writing:

The rule	What it means at your clinic
Consent must be free, specific, informed, unconditional and unambiguous, given by a clear affirmative action.	A tick-box that bundles appointment booking with marketing is not valid consent. Ask for each purpose separately, in plain words.
The patient must get a notice, in English or any Eighth Schedule language, saying what data you collect and why.	Your registration form and WhatsApp opt-in need a short notice in the patient's own language.
Blanket or implied consent for sharing health data is not valid.	If you send patient details to a lab, a TPA or a marketing tool, you need explicit consent for that specific sharing.
Children's data needs verifiable consent from a parent or guardian.	Paediatric clinics must collect a parent's or guardian's consent, not the child's.
Patients can ask to see, correct or erase their data, and to withdraw consent.	Your front desk needs a one-page process for handling these requests.
A data breach must be reported to the Data Protection Board and to affected patients.	Know who in your clinic owns that notification before it is ever needed.

The rule	What it means at your clinic
Penalties can run to hundreds of crores for serious breaches.	The cheapest insurance is simply not storing or sending more patient data than you need.

The NMC ethics rules that touch your marketing

Regulation	In practice
Reg. 6.1 - no soliciting patients, directly or indirectly; no advertising that invites attention to your skill or achievements.	Health-awareness posts are fine. "Best dentist in town" is not. Announcements are permitted only on the seven listed occasions, such as starting practice or changing address.
Reg. 6.1.2 - a self-photograph or publicity material on your letterhead or signboard is self-advertisement.	A clinical photo of the clinic is fine. Your own portrait as branding on the signboard is not.
Reg. 7.11 - do not write lay-press articles or give interviews about diseases that advertise you.	You may write under your own name on public health and hygienic living. Keep it educational, not promotional.
Reg. 7.12 - an institution you run may be advertised in the lay press, but only with the name, type of patients, type of training, facilities and fees.	No superlatives, no outcome claims, no before-and-after guarantees.
Reg. 1.4.1 - display your registration number in the clinic and on every prescription, certificate and money receipt.	Add it to your WhatsApp Business profile, email signature and website too - the Telemedicine Guidelines require it on electronic communication as well.
Reg. 1.3 - keep records of indoor patients for at least three years and issue requested records within 72 hours.	An AI can help you summarise a record. It must never be the place the record lives.
Reg. 1.5 - prescribe drugs with generic names, legibly and preferably in capital letters.	Use AI to format and check legibility, never to add a brand name you did not choose.

The Telemedicine Guidelines, 2020: what they mean for WhatsApp

Most Indian clinics now run on WhatsApp. The Telemedicine Practice Guidelines treat chat and messaging platforms as a valid mode of telemedicine consultation, which brings four practical duties with it:

- Consent. If the patient starts the conversation, consent is implied. If your clinic starts it, you need explicit consent, recorded in the patient's notes.
- Identity. Both sides must be identifiable. You confirm the patient's identity, and the patient must be able to verify your credentials and registration number.
- Display. Your registration number must appear on your prescriptions, your website and your electronic communication, including WhatsApp and email.
- Records. Keep a log of the interaction and the prescription records, the same way you would for an in-person visit.

The safe-words table

When you brief an AI - or edit its output - run the wording through this filter. The left column is the kind of phrase that creates a regulatory problem. The right column says the same thing safely.

Instead of this	Write this
"The best orthopaedic clinic in Delhi"	"An orthopaedic clinic in Delhi with a focus on joint replacement"
"We cure back pain in 3 sessions"	"A structured physiotherapy programme for chronic back pain"
"India's most trusted dermatologist"	"A dermatology practice serving [area] since [year]"
"Book now - 50% off for the first 100 patients"	"Consultation charges are listed on our website"
"Guaranteed results"	"Outcomes vary; your doctor will discuss what to expect"
"Sorry your diabetes treatment here did not work" (public reply)	"We are sorry to read about this experience. Please contact our clinic manager so we can look into it."
"Rate us 5 stars and get a free check-up"	"If you found your visit helpful, a Google review helps other patients find us."

A one-page AI-use policy for your clinic

Write this down, date it, and have every doctor and staff member who uses AI read it. It takes ten minutes and it answers the question a regulator or a patient will eventually ask.

Clinic AI-Use Policy (adapt and adopt)

1. Purpose. AI assistants are used only to draft non-clinical communication, administrative text and patient-education material.

2. No patient identifiers. No name, phone number, address, ABHA number, date of birth, photograph or any other identifier is entered into any AI tool.

3. No clinical decisions. AI output never determines a diagnosis, a prescription or a treatment plan. A registered medical practitioner reviews and owns every output before it reaches a patient.

4. No direct patient counselling. Patients are never counselled or prescribed to by an AI system, in line with the NMC Telemedicine Practice Guidelines, 2020.

5. Review before sending. Every AI-drafted message, post or document is read and edited by a doctor or the clinic manager before it is published or sent.

6. Consent. Patient data is processed only for the purpose the patient agreed to, under the DPDP Act 2023.

7. Records. AI-drafted clinical documents are stored in the patient's record in the clinic's own system, not in the AI tool.

8. Owner. [Name and role] is responsible for this policy and reviews it every six months.

1. Patient Communication

The messages a clinic sends between consultations decide whether a patient trusts you before they arrive. These twelve prompts cover the conversations that go wrong most often: the worried after-hours message, the cost question, the request for antibiotics you will not write, and the angry message you must answer without confirming anything.

1.1 After-hours reply to a worried parent

Use it when: A parent messages at 9 p.m. about a child's fever. You cannot examine them now.

COPY-PASTE PROMPT

You are the front-desk manager of a paediatric clinic in [city]. Draft a WhatsApp reply to a parent who has messaged after hours about their [age]-year-old child with fever for [number] days, no rash, no convulsions, drinking fluids normally. The clinic opens at [time]. Rules: under 110 words, calm and warm, no diagnosis, name no medicine, do not promise the child is fine. Include the standard red-flag advice to go to the nearest emergency department if the child becomes drowsy, refuses fluids, has a fit or breathes fast. End with the exact opening time and a request to call when they open.

Watch out: The red-flag list is what makes this safe. Never send a fever message without it - it protects both the child and your registration.

1.2 Explaining a diagnosis in one calm message

Use it when: You have diagnosed a chronic condition and want the patient to leave with the right words.

COPY-PASTE PROMPT

You are a [specialty] doctor in India. Write a short WhatsApp message that explains a diagnosis of [condition] to an adult patient in plain, everyday English. Structure: one sentence naming the condition simply; one sentence on what it means day to day; two or three sentences on the first practical steps; one sentence reassuring them that the condition is manageable with follow-up. No medical jargon, no statistics, no medicine names, no promises about outcomes. Under 140 words.

Watch out: Ask the AI to also give a version in the patient's own language - Section 11 has the prompt for that.

1.3 Replying when a patient asks, "Is this serious?"

Use it when: You cannot answer a severity question honestly over chat, but you cannot ignore it either.

COPY-PASTE PROMPT

You are a doctor's assistant at a [specialty] clinic. A patient has messaged asking "Is this serious?" about a symptom they described earlier as [brief, non-identifying description]. Draft a reply that: thanks them for asking; explains honestly that whether something is serious depends on an examination, which cannot be done over chat; gives one clear next step (a consultation slot or an in-person review); and states the emergency red flags that should send them to a hospital immediately. Under 120 words. Do not reassure them that it is not serious, and do not alarm them.

Watch out: The honest answer - "this needs an examination" - is also the compliant answer. Never let an AI talk you into a remote all-clear.

1.4 Pre-surgery reassurance message

Use it when: A patient is anxious two days before a planned procedure.

COPY-PASTE PROMPT

You are a [specialty] surgeon in India. Write a WhatsApp message to a patient two days before a planned [procedure] to reduce anxiety. Cover: what to bring, what to stop eating and drinking and when, who to call with questions, and one sentence on what the day will feel like. Warm, unhurried, under 160 words. Do not describe risks, success rates or outcomes. Do not use the words "safe" or "guaranteed".

Watch out: Preparation instructions reduce cancellations more than reassurance does. Keep the medical detail for the consent conversation.

1.5 Answering a cost question without quoting a number

Use it when: A prospective patient asks the fee before booking.

COPY-PASTE PROMPT

You are the practice manager of a [specialty] clinic in [city]. A prospective patient has messaged asking how much a [consultation/procedure] costs. Draft a reply that: welcomes the enquiry; explains that the fee depends on [what it actually depends on - e.g. the type of consultation, whether tests are needed]; gives the standard consultation fee if one exists; offers to share the full, itemised charges before any treatment begins; and invites them to book. Under 110 words, no discounts, no price comparisons with other clinics.

Watch out: Announcing your fees openly is permitted - Regulation 6.1 lists public declaration of charges as an allowed announcement. Haggling in public is not.

1.6 Turning a confusing lab report into a next-step message

Use it when: A patient has sent a photo of a report and asked what it means.

COPY-PASTE PROMPT

You are a [specialty] doctor's assistant. A patient has shared a lab report showing [marker] at [value/range] and is anxious. Write a WhatsApp message that: says the report has been noted; explains that a single value must be read alongside the patient's history and an examination; states clearly what the next step is (a review appointment or a repeat test); and avoids any interpretation of the number itself. Under 100 words. Do not tell the patient whether the value is normal, high or low.

Watch out: Interpreting a result over WhatsApp is practising medicine remotely. Route it to a review appointment instead.

1.7 Seven-day follow-up check-in

Use it when: You want to catch a problem early without sounding like a marketing message.

COPY-PASTE PROMPT

You are a [specialty] clinic in India. Write a WhatsApp follow-up message to a patient seven days after [treatment/consultation]. It should: greet them by first name only; ask how they are feeling in one line; remind them of the one thing they were asked to do (for example, complete the course or return for a review); and give the clinic's contact for anything urgent. Under 90 words, warm and specific, no marketing, no new advice.

Watch out: This is the message that catches a complication before it becomes a complaint. Send it consistently, not selectively.

1.8 Message for a patient who missed an appointment

Use it when: A no-show, handled well, is a recovered patient rather than a lost one.

COPY-PASTE PROMPT

You are the front desk of a [specialty] clinic in [city]. Write a WhatsApp message to a patient who missed their [time] appointment on [day]. Tone: understanding, never scolding. Include: a check that they are alright; an offer of two alternative slots; and one line explaining that if they no longer need the appointment, letting the clinic know frees the slot for someone else. Under 100 words.

Watch out: Never imply the patient wasted your time. The goal is one rebooked slot, not an apology.

1.9 Explaining why you are referring them to a specialist

Use it when: A referral can feel like rejection unless you frame it correctly.

COPY-PASTE PROMPT

You are a [specialty] doctor in India. Write a short WhatsApp message to a patient explaining that you are referring them to a [specialist type] for [reason in plain words]. It must make clear that this is a normal, considered step and not a sign that their case is hopeless; that you remain their doctor for [what you will continue to handle]; and what to carry to the specialist. Under 120 words, reassuring and factual.

Watch out: Patients drop off after referrals when the reason is unclear. One clear message protects continuity of care.

1.10 De-escalating an angry patient message

Use it when: A patient writes in capitals about a bill, a wait or a rude interaction.

COPY-PASTE PROMPT

You are the patient-relations manager of a clinic in India. A patient has sent an angry WhatsApp message saying [short, non-identifying summary of the complaint]. Draft a reply that: acknowledges the experience without confirming any clinical detail; does not argue or explain publicly; apologises for the experience (not for a clinical outcome); gives a named role and a direct channel to resolve it offline; and states a response time. Under 120 words, calm, professional, no defensiveness.

Watch out: Acknowledge the experience, never the medical facts. "We are sorry to read this" is safe; "we are sorry your treatment failed" is a disclosure.

1.11 Refusing antibiotics you will not prescribe

Use it when: The patient is certain they need them; you are certain they do not.

COPY-PASTE PROMPT

You are a [specialty] doctor in India. Write a WhatsApp message explaining, kindly and firmly, that you are not prescribing an antibiotic for [condition] because [plain reason - e.g. it appears to be viral]. It should: validate the patient's discomfort; explain why an antibiotic is not the right tool here; state what you are recommending instead and why; and offer a review if things change. Under 130 words, respectful, no lecturing, no jargon.

Watch out: Pair this with the antibiotic-awareness education prompt in Section 2 so the explanation does not have to be repeated every time.

1.12 Discharge-day instructions in plain words

Use it when: The patient is going home and will forget half of what was said.

COPY-PASTE PROMPT

You are a [specialty] doctor in India. Convert the following clinical discharge points into a plain-language WhatsApp message for an adult patient: [paste your raw bullet points, with no identifiers]. Format: a one-line summary; a numbered list of what to do; a numbered list of what to avoid; the warning signs that mean "come back immediately"; and the follow-up date. Use short sentences and everyday words. Do not add any instruction that is not in my notes.

Watch out: The last line matters - it stops the AI inventing advice. Always add it.

2. Patient Education

Educated patients follow treatment, complain less and refer more. These prompts turn clinical knowledge into material a patient can actually read - in plain words, with the danger signs spelled out, and without a single invented fact.

2.1 A diabetes diet guide in plain language

Use it when: Your newly diagnosed patients keep asking the same food questions.

COPY-PASTE PROMPT

You are a [specialty] doctor in India writing patient-education material. Produce a one-page, plain-language guide on eating with type 2 diabetes for an Indian patient. Cover: how to build a plate at a typical Indian meal; three swaps the patient can make this week (for example, [swap]); what to limit rather than ban; and how to read a food label. Use everyday Indian food examples. No calorie counts, no numbers I have not given you, no medicine advice. Keep sentences short. End with "ask your doctor before changing your diet".

Watch out: Ask for the same guide at a 6th-grade reading level for patients with low literacy, using the prompt in Section 11.

2.2 Hypertension explained for a first-time patient

Use it when: A new diagnosis lands better with a simple, calm explainer.

COPY-PASTE PROMPT

You are a [specialty] doctor in India. Write a short patient handout explaining high blood pressure to someone newly diagnosed. Cover: what the two numbers mean in one line each; why it usually has no symptoms; why treatment still matters; the daily habits that help most; and when to seek urgent help. Around 300 words, plain English, no scare tactics, no specific drug names, no target numbers I have not provided.

Watch out: Never let the AI supply a blood-pressure target. Give it yours, or leave it out.

2.3 Post-operative care instructions for any surgery

Use it when: One reusable template for the common procedures you perform.

COPY-PASTE PROMPT

You are a [specialty] surgeon in India. Write a post-operative care sheet for a patient recovering from [procedure]. Sections: wound care; pain and what is normal; activity and rest; diet; medicines only as prescribed by me; warning signs needing immediate attention; and the review schedule. Plain language, short sentences, one page. Do not add any medicine name or dose - leave a blank for me to fill in.

Watch out: The blank is deliberate. It keeps prescribing with you and off the AI's page.

2.4 Fever and when to worry, for parents

Use it when: The single handout that prevents both panic and dangerous delay.

COPY-PASTE PROMPT

You are a paediatrician in India. Write a parent-facing guide to fever in children. Cover: what counts as fever; how to measure it; what to do at home; the red-flag signs that mean go to hospital now (list at least six); and why antibiotics are not always needed. Around 320 words, warm and calm, no medicine names, no doses, no reassurance that any specific case is safe.

Watch out: Red-flag lists save lives and protect you. Keep at least six items and never trim them for length.

2.5 Pregnancy diet and danger signs

Use it when: Antenatal counselling, written down so it is remembered.

COPY-PASTE PROMPT

You are an obstetrician in India. Write a plain-language antenatal handout covering: a healthy daily diet during pregnancy with Indian food examples; foods to avoid; the importance of iron and folic acid as prescribed by me; and the danger signs in pregnancy that require immediate hospital care. Around 340 words, gentle and clear, no medicine names beyond the supplements I mentioned, no reassurance about any individual pregnancy.

Watch out: Danger-sign sections are the highest-value part of any antenatal handout. Keep them prominent, not buried.

2.6 Explaining a chronic condition to a caregiver

Use it when: The person who manages the medicines is often not the patient.

COPY-PASTE PROMPT

You are a [specialty] doctor in India. Write a short guide for a family caregiver of a patient with [condition]. Cover: what the condition is in one paragraph; what the caregiver should watch for day to day; how to support the patient's routine; what to record between visits; and when to call the clinic or go to hospital. Plain language, around 300 words, no medicine names, no doses, no outcome promises.

Watch out: Caregiver material improves adherence more than patient material in many Indian households.

2.7 Asthma inhaler technique, explained simply

Use it when: Most patients use inhalers incorrectly and no one has told them.

COPY-PASTE PROMPT

You are a [specialty] doctor in India. Write step-by-step instructions for correct use of a metered-dose inhaler, in plain language a first-time patient can follow. Include: preparing the inhaler; the breathing technique; rinsing the mouth after; cleaning the device; and what to do if the patient still feels breathless. Around 280 words, numbered steps, no medicine names, no doses.

Watch out: Ask the AI to add a line telling the patient to demonstrate the technique at their next visit.

2.8 Antibiotic awareness message

Use it when: One reusable message you can send to every patient who asks for antibiotics.

COPY-PASTE PROMPT

You are a [specialty] doctor in India. Write a patient-facing explainer on antibiotic resistance. Cover: what antibiotics do and do not treat; why finishing a prescribed course matters; why they are not useful for viral illnesses like most colds and flu; and what to do instead when symptoms are mild. Around 260 words, plain and non-judgemental, no medicine names, no doses, no shaming language.

Watch out: Non-judgemental wording gets read. Preachy wording gets ignored.

3. Appointments & Follow-up

Reminders cut no-shows, and no-shows are pure lost revenue and lost continuity. These prompts build the short, clear messages that keep your schedule full and your patients prepared.

3.1 Appointment confirmation with preparation

Use it when: Booked patients need to know what happens next, and what to bring.

COPY-PASTE PROMPT

You are the front desk of a [specialty] clinic in [city]. Write a WhatsApp appointment confirmation for [day, time]. Include: the clinic address with one landmark; what to bring (previous prescriptions, reports, ID); whether to come fasting or with a full bladder if relevant; the approximate duration; and the rescheduling number. Under 120 words, friendly and scannable, use line breaks.

Watch out: Landmarks beat map links in many parts of India. Include both.

3.2 48-hour reminder with a checklist

Use it when: The reminder that actually reduces no-shows.

COPY-PASTE PROMPT

You are a clinic in India. Write a WhatsApp reminder 48 hours before an appointment for [purpose]. Format: one line confirming date and time; a short checklist of what to bring; one line on how long the visit usually takes; and a request to reply CONFIRM or RESCHEDULE. Under 100 words, use a numbered list.

Watch out: Asking for a one-word reply gets a response. Asking for a paragraph does not.

3.3 Four-hour reminder with travel details

Use it when: A same-day nudge, with the practical details that stop a late arrival.

COPY-PASTE PROMPT

You are a clinic in [city]. Write a short WhatsApp message four hours before an appointment. Include: the time; a one-line travel note if the area is congested; the parking or drop-off option; the number to call if running late; and a friendly line that arriving a few minutes early helps. Under 80 words.

Watch out: This is the message that turns a 30-minute delay into a 5-minute one.

3.4 No-show recovery message

Use it when: One gentle message recovers a meaningful share of missed slots.

COPY-PASTE PROMPT

You are the front desk of a clinic in India. Write a WhatsApp message to a patient who missed a [time] appointment on [day]. It should: check that they are well; avoid any blame; offer two specific alternative slots; and add one line that if they no longer need the appointment, letting the clinic know helps another patient. Under 100 words, warm.

Watch out: Offer two concrete slots, not an open invitation. Concrete choices convert.

3.5 Annual preventive health-check reminder

Use it when: A recall that adds value instead of sounding like a sales pitch.

COPY-PASTE PROMPT

You are a [specialty] clinic in India. Write a WhatsApp recall message to a patient due for an annual preventive health check, last seen [period] ago. It should: remind them gently why an annual review matters; list in plain words what a standard check usually covers; make clear it is optional and they can ask questions first; and offer to book. Under 120 words, no scare language, no discount, no package pricing pressure.

Watch out: Preventive recall is one of the few genuinely patient-serving marketing messages. Keep it factual.

3.6 Paediatric vaccination due reminder

Use it when: Parents miss vaccination dates; a good reminder fixes that.

COPY-PASTE PROMPT

You are a paediatric clinic in India. Write a WhatsApp reminder to a parent that their child's [vaccine, as per the national schedule] is due around [age/date]. Include: a plain one-line explanation of what the vaccine protects against; the importance of staying on schedule; the two clinic slots available; and what to bring (the vaccination card). Under 110 words, reassuring, no medicine names beyond the vaccine, no pressure.

Watch out: Always name the vaccination card. It is the one thing parents forget.

3.7 Pre-procedure fasting instructions

Use it when: A clear, timed instruction sheet prevents a cancelled procedure.

COPY-PASTE PROMPT

You are a [specialty] clinic in India. Write plain-language pre-procedure instructions for a patient scheduled for [procedure] at [time]. Cover: when to stop eating solid food; when to stop clear fluids; which regular medicines to continue or pause, leaving a blank for me to fill; what to bring; and whom to call with questions. Numbered steps, around 200 words, no medicine names I have not given you.

Watch out: Leave the medicine decision blank. Fasting rules are safe to template; drug holds are not.

3.8 Post-operative review booking message

Use it when: Booking the review at the right moment keeps the patient on track.

COPY-PASTE PROMPT

You are a [specialty] clinic in India. Write a WhatsApp message to a patient recovering from [procedure], offering to book their post-operative review for around [day/period]. Include: why the review matters; what the doctor will check; roughly how long it takes; and two slot options. Under 110 words, warm and practical.

Watch out: Send this the day the patient is discharged, not a week later.

4. Google Business Profile & Local Discovery

Roughly 68 per cent of first-time patient bookings in Indian metros begin with a Google Maps search. Your Google Business Profile is therefore the highest-leverage page your clinic owns - and it is free. These prompts build it, correctly.

4.1 A compliant Google Business Profile description

Use it when: The 750-character field most clinics fill badly.

COPY-PASTE PROMPT

You are a healthcare marketing writer in India. Write a Google Business Profile description, under 750 characters, for a [specialty] clinic in [area, city]. It must: say who the clinic serves; list the main services in plain words; mention the years of experience and the registration context if I provide it; and end with a line inviting patients to call for an appointment. No superlatives, no "best", no outcome claims, no prices, no promotional offers. Plain, factual, warm.

Watch out: Google's own guidance says trust matters most for health content. Factual beats boastful every time.

4.2 A plain-words services list

Use it when: Patients search in everyday language, not clinical language.

COPY-PASTE PROMPT

You are a healthcare SEO writer in India. For a [specialty] clinic, produce a list of 12 to 18 services written the way patients actually search for them, in plain words, with the clinical term in brackets after each. Example: "Pregnancy care and delivery (obstetrics)". Keep each under eight words. No promotional language.

Watch out: Put the patient's words first and the clinical term second. That matches how people type.

4.3 A Google Business Profile post for a health-awareness day

Use it when: Regular posts signal an active profile without advertising yourself.

COPY-PASTE PROMPT

You are a healthcare content writer in India. Write a Google Business Profile post for [awareness day, e.g. World Diabetes Day] for a [specialty] clinic. It should be educational: two or three plain-language facts or tips, one line on when to see a doctor, and a neutral call to action ("Call the clinic to book a consultation"). Under 250 characters for the main text, plus a short headline. No brand superlatives, no offers, no patient stories.

Watch out: Awareness days are the safest recurring content a clinic can post. Educational, not promotional.

4.4 Seed questions for your profile's Q&A

Use it when: Pre-empt the questions your front desk answers fifty times a week.

COPY-PASTE PROMPT

You are a clinic operations writer in India. Write ten realistic patient questions and short, compliant answers for the Google Business Profile Q&A section of a [specialty] clinic in [city]. Questions should cover timings, location, appointment process, what to bring, whether walk-ins are accepted, and general service questions. Answers under 40 words each, factual, no medical advice, no prices unless I provide them.

Watch out: The Q&A section is public and permanent. Keep every answer free of medical advice.

4.5 A local keyword set for your specialty and city

Use it when: The exact phrases to use in your profile, website and posts.

COPY-PASTE PROMPT

You are a local SEO specialist for healthcare in India. For a [specialty] clinic in [city and area], produce a list of 25 search phrases a real patient might type - mixing English and the local language transliteration, long-tail and short-tail, symptom-led and service-led. Group them under headings: symptom searches, service searches, location searches, and comparison searches. Plain list, no commentary.

Watch out: Symptom-led phrases - "knee pain doctor near me" - often convert better than service-led ones.

4.6 Clinic photo captions and alt text

Use it when: Accessible, accurate captions for the photos Google rewards.

COPY-PASTE PROMPT

You are a healthcare content writer in India. For a [specialty] clinic, write captions and image alt-text for eight photos: the entrance, the waiting area, the consultation room, the reception, the equipment, the team (no patients), the parking or drop-off, and the signboard. Each caption under 15 words, each alt-text under 20 words, descriptive and factual. No patient faces, no patient records visible.

Watch out: Never upload a photo with a patient or a screen showing patient data. That is a data-disclosure risk.

4.7 A ten-minute monthly profile audit

Use it when: The checklist that keeps your listing accurate and unsuspended.

COPY-PASTE PROMPT

You are a clinic operations manager in India. Create a ten-minute monthly checklist to audit a Google Business Profile for a [specialty] clinic. Include checks on: name exactly as on the signboard; category and secondary categories; hours including festival special hours; a phone number that is actually answered; the services list; recent photos; replies to all new reviews; and the performance numbers. Present as a numbered checklist with a one-line note on what to fix if each item is wrong.

Watch out: A wrong festival-hours entry sends patients to a closed shutter and loses the booking permanently.

4.8 A search-intent map for your area

Use it when: Understand what your neighbours are actually searching before you write anything.

COPY-PASTE PROMPT

You are a local healthcare marketing analyst in India. For a [specialty] clinic in [area, city], map the likely search intent of patients into four groups: emergency and urgent; symptom-led research; service comparison; and ongoing-care management. For each group, list three example searches, the kind of content that answers them, and the one action you want the searcher to take. Present as a table. No invented search volumes.

Watch out: Do not let the AI fabricate search volumes. Ask it for intent and content, not numbers.

5. Reviews & Reputation, the Safe Way

Only about 60 per cent of Indian healthcare reviews get a reply, and the replies that do appear are where clinics most often breach patient confidentiality without realising it. These eight prompts get you the reputation benefit without the compliance risk.

5.1 A review request message with no incentive and no health detail

Use it when: The single most common compliance mistake is the review-request message.

COPY-PASTE PROMPT

You are the practice manager of a [specialty] clinic in India. Write a WhatsApp message asking a patient to leave a Google review after their visit. Rules: ask every patient the same way; no incentive of any kind; no mention of the patient's condition, treatment or visit type; no pressure; and a clear statement that it is entirely optional. Include a placeholder for the review link. Under 70 words, warm and light.

Watch out: Google and Practo both forbid incentives and selective soliciting. A free check-up for a five-star review can cost you your profile.

5.2 A front-desk script for sharing the review link

Use it when: Staff need one consistent line, said the same way to everyone.

COPY-PASTE PROMPT

You are a clinic manager in India. Write a short front-desk script that staff can use when handing a patient a review QR code or link at the end of a visit. It must: be said to every patient the same way; be neutral about the expected rating; mention no condition or treatment; and take under fifteen seconds to say. Provide three phrasings - English, Hindi and one other Indian language.

Watch out: Consistency is the compliance defence. If you asked only the happy patients, you cannot prove you did not.

5.3 Reply to a five-star review

Use it when: Warm, brief, and it never confirms who the reviewer is.

COPY-PASTE PROMPT

You are the practice manager of an Indian clinic. Write a public reply to a five-star Google review. The reviewer may have mentioned their treatment; your reply must not repeat or confirm any treatment, condition or patient relationship. Thank them for their kind words about the clinic and the team, and stop. Under 40 words. Provide an English version and a Hindi version.

Watch out: Notice what you must leave out: "glad your treatment went well", "see you at your follow-up", "wishing you a quick recovery". Each confirms a patient relationship.

5.4 Reply to a one-star review

Use it when: The hardest reply in any clinic, because the instinct is to explain.

COPY-PASTE PROMPT

You are the patient-relations manager of an Indian clinic. Write a public reply to a one-star Google review in which the reviewer describes a bad experience, possibly naming a treatment. Rules: do not confirm they were a patient; do not repeat or acknowledge any clinical detail; do not apologise for a specific clinical outcome; acknowledge the experience in general terms; give a named role and a direct offline channel; state a response time. Under 80 words, calm and professional.

Watch out: The offline channel is where specifics are safe, because the patient identifies themselves and gives consent there.

5.5 Reply to a review that names a treatment

Use it when: The reviewer volunteered the detail. That does not license you to confirm it.

COPY-PASTE PROMPT

You are a clinic manager in India. A patient has left a review naming the exact treatment they received and asking others to visit. Write a public reply that thanks them without confirming, repeating or commenting on any treatment detail. Keep it warm and generic about the clinic and the team. Under 45 words. Add one line I could use if I later want to invite them to share feedback privately.

Watch out: Even when the reviewer says it first, your confirmation is a fresh disclosure under the DPDP Act.

5.6 Reply to a review alleging a wrong diagnosis

Use it when: You cannot defend yourself publicly without disclosing.

COPY-PASTE PROMPT

You are the medical director's office at an Indian clinic. Write a public reply to a review alleging a wrong diagnosis. Rules: no confirmation of the patient relationship; no discussion of the case; a brief, sincere expression of concern about the experience; an invitation to contact a named role through a direct channel; and a statement that the clinic takes every concern seriously. Under 80 words, dignified and non-defensive. Do not use the phrase "we are sorry your treatment did not work".

Watch out: For genuinely defamatory reviews, escalate through Google's process and counsel - do not argue in public. Public engagement rewards the poster with reach.

5.7 An internal SOP for contesting a fake review

Use it when: Fake and competitor reviews need a process, not a mood.

COPY-PASTE PROMPT

You are a healthcare marketing operations writer in India. Write a short internal SOP for handling a suspected fake or policy-violating Google review at a clinic. Cover: who assesses it; the evidence to gather; how to report it under Google's policies; the timeline to expect; what to do if it is not removed; and what the clinic must never do (write its own reviews, offer incentives, pressure the reviewer). Present as numbered steps with an owner for each.

Watch out: Never write reviews for your own practice. Google can stop the profile receiving new reviews entirely.

5.8 A monthly reputation report prompt

Use it when: Turn a pile of reviews into three actions.

COPY-PASTE PROMPT

You are a healthcare reputation analyst in India. I will paste anonymised review text from the past month. Analyse it and produce: the top three complaint themes with counts; the top three praise themes; the two replies that should be improved and how; and three specific operational actions for the clinic this month. Do not reproduce any review verbatim. Present as a short report under 400 words.

Watch out: Research on 245 Indian healthcare brands found staff and bedside manner drove 23 per cent of negative reviews, with wait times second at 18.8 per cent. Watch for those two first.

6. Clinic Content & Social Media

Content keeps a clinic visible between visits - as long as it educates rather than advertises. These prompts produce the safe, useful kind: myth-busters, awareness-day posts, and reels a patient would actually watch.

6.1 An Instagram carousel: myth versus fact

Use it when: The format that performs best for clinics and stays clearly educational.

COPY-PASTE PROMPT

You are a healthcare content writer in India. Create a six-slide Instagram carousel for a [specialty] clinic on [topic]. Slide 1: a myth in the patient's own words. Slides 2 to 5: one fact each, in plain language, with one short reason. Slide 6: when to see a doctor, and a neutral call to action. Each slide under 25 words. No medicine names, no guarantees, no patient stories, no before-and-after claims.

Watch out: Myth-buster carousels are educational by design, which keeps them clear of the advertising rules.

6.2 A 30-day content calendar for a clinic

Use it when: One month of posts, planned in ten minutes.

COPY-PASTE PROMPT

You are a healthcare social-media planner in India. Build a 30-day content calendar for a [specialty] clinic's Instagram and Google Business Profile. Mix: 8 myth-busters, 6 awareness-day or seasonal posts, 5 patient-education explainers, 4 behind-the-scenes clinic posts (no patients), 4 appointment-reminder posts, and 3 community or festival posts. For each day give: the format, the topic, and a one-line hook. No promotional offers, no superlatives.

Watch out: Plan around the Indian calendar - the monsoon illness season, exam stress, festival eating, and the winter air-quality months.

6.3 A 30-second patient-education reel script

Use it when: Short, useful video is the most-shared clinic content.

COPY-PASTE PROMPT

You are a healthcare video scriptwriter in India. Write a 30-second reel script for a [specialty] doctor on [topic]. Structure: a three-second hook; the one thing the patient must understand; one practical tip; and a closing line inviting questions at a consultation. Provide shot notes (who is on camera, any on-screen text) and a spoken script under 75 words. No diagnosis, no medicine names, no outcome promises.

Watch out: Film in the clinic, with the doctor in professional attire and the registration number visible on screen.

6.4 A festival or awareness-day post

Use it when: Seasonal relevance without seasonal hype.

COPY-PASTE PROMPT

You are a healthcare content writer in India. Write a short social post for a [specialty] clinic for [Diwali / Holi / World Heart Day / exam season]. It should be genuinely useful - for example, tips on safe sweets for a patient with diabetes, or eye safety during Holi. Include: a warm one-line greeting; two or three practical tips; and a line on when to seek medical help. Under 120 words. No offers, no discounts, no brand superlatives.

Watch out: A useful festival post builds far more goodwill than a discount.

6.5 A patient-story post without revealing identity

Use it when: The most-misused format in healthcare marketing.

COPY-PASTE PROMPT

You are a healthcare content writer in India. Write a social post that shares the lesson from a patient's recovery without identifying them in any way. Rules: no name, age, gender, location, occupation or any detail that could identify the person; no photograph; no before-and-after; no specific treatment claims; frame it as a general education point. Provide a short post under 100 words and a one-line caption. Add a line asking readers to consult a doctor for their own case.

Watch out: The safest rule for patient stories in India is simple: if a reader could recognise the person, do not publish it.

6.6 A doctor bio for the clinic website, NMC-compliant

Use it when: A professional profile that informs without advertising.

COPY-PASTE PROMPT

You are a healthcare writer in India. Write a website bio for a doctor at a [specialty] clinic. Include: full name and recognised degrees as suffixes only; role at the clinic; areas of practice; years of experience; registration context if I provide it; and one line on the clinic's approach to care. No superlatives, no "best", no claims of being a leader or pioneer, no patient outcome claims, no personal photograph as branding. Around 120 words.

Watch out: Regulation 6.1.2 treats a self-photograph as publicity material. A professional headshot on an 'about' page is common practice, but keep it off the signboard and letterhead.

6.7 A blog-post outline: when to see a specialist

Use it when: The article that quietly captures high-intent search traffic.

COPY-PASTE PROMPT

You are a healthcare content writer in India. Create a blog-post outline for a [specialty] clinic titled "When should you see a [specialist]?". Include: a short intro; a list of common symptoms that warrant a visit; what happens at a first consultation; questions the patient should ask; and a closing line inviting them to book. Provide H2 and H3 headings and two sentences of guidance under each. Educational only, no diagnosis, no treatment claims.

Watch out: Publish under a clinic byline rather than a doctor's personal name to stay clear of Regulation 7.11.

6.8 A WhatsApp status tip of the week

Use it when: The cheapest recurring content your clinic has.

COPY-PASTE PROMPT

You are a healthcare content writer in India. Write four short "tip of the week" posts for a [specialty] clinic's WhatsApp status - one each for [topic 1], [topic 2], [topic 3], [topic 4]. Each should be a single practical tip in under 30 words, plain language, no medicine names, no diagnosis, and end with a neutral line such as "Consult your doctor for your own case."

Watch out: Four posts, one a week, is a month of content from a single prompt.

7. OPD Admin & Documentation

This is where AI saves the most doctor-hours - and where the rules are strictest. These prompts help you draft, format and summarise. They never let the AI hold the record or make the clinical call.

7.1 Structure an OPD note from your bullet points

Use it when: You have scribbled the essentials; you want a clean, standard note.

COPY-PASTE PROMPT

You are a medical documentation assistant in India. Convert my rough bullet points into a clean, standard OPD consultation note with these headings: chief complaint; history; examination findings; diagnosis; plan; and follow-up. Do not add, infer or invent any clinical detail. Where my notes are incomplete, insert [TO COMPLETE] rather than guessing. Here are my notes: [paste, with no patient identifiers].

Watch out: The [TO COMPLETE] rule is essential. An AI that fills gaps in a medical record is a liability.

7.2 Write a discharge summary from raw notes

Use it when: A structured summary, fast, from the notes you already wrote.

COPY-PASTE PROMPT

You are a medical documentation assistant in India. Draft a discharge summary from my notes, with these sections: admission and discharge dates; presenting complaint; key findings; procedures done; treatment given; condition at discharge; medicines to continue (leave the actual list blank for me); and follow-up instructions. Use only the information I provide; insert [TO COMPLETE] for anything missing. Here are my notes: [paste, with no identifiers].

Watch out: The medicine list stays blank on purpose. Prescribing is yours alone.

7.3 Draft a medical certificate with the required fields

Use it when: Certificates are a compliance document, not a formality.

COPY-PASTE PROMPT

You are a medical documentation assistant in India. Draft a medical certificate for [purpose, e.g. sick leave / fitness to resume work] with all the fields required by the Indian Medical Council (Professional Conduct, Etiquette and Ethics) Regulations, 2002: patient identification marks, signature or thumb impression, address, at least one identification mark, the diagnosis or statement as appropriate, dates, and the doctor's name, designation and registration number. Leave clinical fields as [TO COMPLETE] for me. Do not invent any clinical fact.

Watch out: Regulation 1.3.3 requires identification marks and a copy kept on file. Check every certificate for these.

7.4 Summarise a patient's history for a new colleague

Use it when: A handover summary that respects the record.

COPY-PASTE PROMPT

You are a medical documentation assistant in India. Summarise the following case notes into a one-page handover for a colleague who is taking over care: relevant history; current diagnosis; current plan; what has changed recently; and what to watch for. Use only what I provide; mark gaps as [TO COMPLETE]. Do not suggest a diagnosis or a change in treatment. Here are the notes: [paste, with no identifiers].

Watch out: A handover is a clinical document. The AI formats it; the doctor owns it.

7.5 Clean up dictation into a consultation note

Use it when: You dictated on the move and it came out messy.

COPY-PASTE PROMPT

You are a medical documentation assistant in India. Clean up the following dictated consultation note into clear, well-structured prose under standard headings. Fix grammar and order, remove repetition, and keep every clinical fact exactly as dictated. Do not add or infer anything. Mark unclear sections as [TO COMPLETE]. Here is the dictation: [paste, with no identifiers].

Watch out: Read the cleaned note against the recording before signing. Clean-up tools occasionally drop a negative.

7.6 Prepare an insurance or TPA summary letter

Use it when: A clear, structured letter speeds up every claim.

COPY-PASTE PROMPT

You are a medical documentation assistant in India. Draft a summary letter to an insurance company or TPA about a patient's hospitalisation, using only the following details: [paste, with no identifiers]. Structure: patient reference (left blank for me); admission and discharge dates; diagnosis; treatment given; procedures; and documents enclosed. Professional tone, under 350 words, no clinical opinion beyond what I provided, no guarantees about the claim.

Watch out: Never let the AI characterise the claim as payable. That is the insurer's call, not the clinic's.

7.7 Create a standard post-operative instruction sheet

Use it when: One reusable template you can adapt for each procedure.

COPY-PASTE PROMPT

You are a medical documentation assistant in India. Create a reusable post-operative instruction sheet template for [specialty] procedures. Sections: wound care; pain; activity; diet; medicines (leave blank); warning signs; and review schedule. Write it in plain patient language with clear headings and short sentences, and leave a [PROCEDURE-SPECIFIC] block for me to fill in each time. Around 350 words.

Watch out: A good template means the instructions go out the same, complete, every time.

7.8 Run a documentation-quality check

Use it when: Find the gaps before a regulator or an auditor does.

COPY-PASTE PROMPT

You are a medical records quality reviewer in India. Review the following OPD note against the NMC requirement to maintain records in a standard proforma, and against the standard headings. List: any missing heading; any internal inconsistency; any illegible or ambiguous phrase; and any missing follow-up instruction. Do not rewrite the note. Here is the note: [paste, with no identifiers].

Watch out: Run this monthly across a sample of notes. It catches drifting documentation habits early.

8. Letters & Professional Writing

Referral letters, case reports and professional profiles are where a doctor's writing is most visible to peers. These prompts make the first draft fast and the format consistent.

8.1 A structured referral letter to a specialist

Use it when: A good referral letter gets the patient seen faster and treated better.

COPY-PASTE PROMPT

You are a medical writing assistant in India. Draft a referral letter from me, a [specialty] doctor, to a [specialist type], about a patient I am referring for [reason in plain words]. Include: a one-line reason for referral; relevant history and findings as I provide them; investigations done; current treatment; and the specific question I am asking the specialist. Use only my information, mark gaps as [TO COMPLETE], and keep it under 400 words, professional and concise.

Watch out: A referral that asks one specific question gets a faster, more useful reply than a vague one.

8.2 A second-opinion summary for a patient

Use it when: Patients seeking a second opinion need a clear, honest summary.

COPY-PASTE PROMPT

You are a medical writing assistant in India. Draft a summary a patient can carry when seeking a second opinion, based on my notes: [paste, with no identifiers]. Include: presenting problem; diagnosis; treatment given so far; current status; and the question the patient wants answered. Plain language, under 350 words, no opinion on what the second doctor will or should say, no criticism of any other clinician.

Watch out: Never criticise another doctor in writing - Regulation 4.3 is explicit on this.

8.3 A medical fitness certificate

Use it when: The document that gets misused, so it must be precise.

COPY-PASTE PROMPT

You are a medical writing assistant in India. Draft a medical fitness certificate for [purpose, e.g. employment / sports / travel], following the format in the Indian Medical Council regulations. Include: my name, designation and registration number; the patient reference field left blank; the examination performed; the fitness statement; the validity period; and place for my signature and date. Mark clinical fields as [TO COMPLETE]. Do not state a fitness opinion I have not given you.

Watch out: A fitness certificate is a legal document. Sign only what you actually examined.

8.4 A sick-leave certificate with proper fields

Use it when: Short, but the fields are not optional.

COPY-PASTE PROMPT

You are a medical writing assistant in India. Draft a sick-leave certificate for [number] days from [date], including: patient name field (blank for me to fill); identification marks field; the nature of illness at the level of detail I provide; the rest advised; the dates; and my name, designation and registration number. Mark clinical fields as [TO COMPLETE]. Do not invent a diagnosis.

Watch out: Regulation 1.3.3 requires identification marks and a retained copy. Both are easy to forget under time pressure.

8.5 A reply to a request for medical records

Use it when: Requests for records have a deadline attached.

COPY-PASTE PROMPT

You are a clinic records officer in India. Draft a professional reply to a request for a patient's medical records from [the patient / an authorised attendant / a legal authority]. The reply should: acknowledge the request; state that records will be provided within 72 hours as required; list the documents being provided; note any fee if applicable; and state the identification required. Under 200 words, formal but clear.

Watch out: Regulation 1.3.2 sets a 72-hour window. An AI-drafted reply keeps you inside it.

8.6 An abstract for a case report or conference

Use it when: Turn your notes into a structured abstract.

COPY-PASTE PROMPT

You are a medical writing assistant in India. Draft a structured abstract for a case report or conference submission on [topic], based on the following clinical details: [paste, with no identifiers]. Sections: background; case presentation; investigations; management; outcome; and conclusion. Under 300 words, neutral academic tone, no invented findings, and no claim of novelty unless I state it. Mark anything unclear as [TO COMPLETE].

Watch out: Do not publish any case detail that could identify the patient. Anonymise before you draft.

8.7 Update your professional profile or CV

Use it when: A clean professional profile, without the advertising traps.

COPY-PASTE PROMPT

You are a professional writing assistant in India. Draft a concise professional profile for a doctor, based on the following facts: [paste - qualifications, roles, experience, areas of practice, publications]. Format: a short opening summary; qualifications; experience; areas of practice; and professional memberships. Around 200 words, factual and restrained, no superlatives, no claims of being the best or a leader, no patient outcome claims.

Watch out: Restraint is not modesty here - it is compliance with the advertising rules.

9. Practice Growth & Retention

Growth for a clinic is mostly retention plus word of mouth. These prompts help you ask patients the right questions, bring lapsed patients back, and launch new services without crossing into soliciting.

9.1 A short patient-feedback survey

Use it when: Ask before the review, and you learn where the problems are.

COPY-PASTE PROMPT

You are a patient-experience consultant in India. Design a five-question feedback survey for a [specialty] clinic, short enough to complete on a phone in under a minute. Cover: ease of booking; wait time; how clearly the doctor explained things; how the staff treated them; and likelihood to recommend. Provide a five-point scale for each and one optional open question. Add a line explaining what the clinic will do with the feedback.

Watch out: Ask about wait time and staff manner first. They are the two biggest sources of negative reviews in India.

9.2 A recall message for lapsed patients

Use it when: A gentle, useful reason to come back - never a guilt trip.

COPY-PASTE PROMPT

You are the practice manager of a [specialty] clinic in India. Write a WhatsApp recall message to a patient who has not visited in [period]. It should: mention that the clinic is checking in; give one genuine, useful reason to return (a review due, a preventive check, a follow-up that was planned); make clear it is entirely optional; and offer to book. Under 100 words, warm, no discount, no pressure, no clinical detail.

Watch out: A genuine clinical reason converts. "We miss you" does not.

9.3 A new-service launch message

Use it when: Announce a service without advertising yourself.

COPY-PASTE PROMPT

You are a clinic communications writer in India. Write a WhatsApp or email message announcing a new service at a [specialty] clinic: [service name and plain description]. Include: what the service is; who it is for; what the patient can expect; and how to book. Under 120 words, informative and calm, no superlatives, no "state-of-the-art", no offers or discounts, no outcome claims.

Watch out: Regulation 6.1 permits a formal announcement of a change in practice. Inform, do not solicit.

9.4 A follow-up programme design

Use it when: Turn one-off visits into a managed care relationship.

COPY-PASTE PROMPT

You are a clinic operations consultant in India. Design a simple follow-up programme for a [specialty] clinic. Specify: which patient groups get a follow-up and when; the three touchpoints (for example, day 7, day 30, day 90) and what each message should achieve; who on staff sends each; and how the clinic will record consent for these messages. Present as a table with group, timing, channel, and owner.

Watch out: Consent for follow-up messages is a DPDP requirement. Record it at registration, not afterwards.

9.5 A transparent price-list communication

Use it when: Patients who know the cost complain less about it.

COPY-PASTE PROMPT

You are a practice manager in India. Draft a short, clear communication of a [specialty] clinic's standard charges for patients: the consultation fee; what it includes; how procedure costs are estimated; what is not included; and how patients can ask for an itemised estimate before treatment. Under 200 words, plain and transparent, no comparisons with other clinics, no offers. Present as a short note patients can read at the desk.

Watch out: Announcing charges is expressly permitted. Hidden charges are what create complaints.

9.6 A clinic FAQ sheet for the front desk

Use it when: One sheet that answers ninety per cent of the calls.

COPY-PASTE PROMPT

You are a clinic operations writer in India. Write a one-page FAQ for the front desk of a [specialty] clinic in [city]. Questions to cover: timings and holidays; how to book; walk-ins; what to bring; whether insurance is accepted; how records are shared; and how to reach the clinic. Answers under 40 words each, factual, no medical advice, no prices unless I provide them.

Watch out: Give every new front-desk hire this sheet on day one.

9.7 A monthly practice review prompt

Use it when: Five questions that show whether the clinic is actually healthy.

COPY-PASTE PROMPT

You are a practice-management consultant in India. Give me a monthly review template for a [specialty] clinic, built around five questions: patient volume and no-show rate; new versus returning patients; review score and reply rate; revenue per consultation; and one operational bottleneck. For each, specify the number to track, where it comes from, and the one action to take if it moves the wrong way. Present as a table.

Watch out: Track the no-show rate every month. It is the quietest and most fixable leak in most Indian clinics.

10. Compliance & AI Safety

These six prompts build the documents that keep your clinic on the right side of the DPDP Act 2023 and the NMC ethics regulations - consent notices, an AI-use policy, and a de-identification habit you can trust.

10.1 A DPDP consent notice for your clinic, in plain language

Use it when: The notice the law requires, written so patients actually understand it.

COPY-PASTE PROMPT

You are a data-protection writer in India, working with a [specialty] clinic. Draft a short patient consent notice under the Digital Personal Data Protection Act, 2023. It must state, in plain words: what personal data the clinic collects; why; who it may be shared with (for example, a lab or an insurer); how the patient can see, correct or erase their data; how to withdraw consent; and how to complain. Keep it under 250 words. Add a one-line version for a WhatsApp opt-in.

Watch out: The Act requires the notice to be available in English or any Eighth Schedule language. Ask for a Hindi version too.

10.2 A consent text for WhatsApp communication

Use it when: One clear sentence at the point of collection prevents a later dispute.

COPY-PASTE PROMPT

You are a data-protection writer in India. Write a short consent line that a [specialty] clinic can add to its patient registration form and its first WhatsApp message, covering: appointment reminders; follow-up messages; and health-education content. It must be a clear affirmative opt-in, separate for each purpose, in plain language, with a one-line note on how to opt out. Provide English and Hindi versions.

Watch out: Do not bundle. The DPDP Act requires consent to be specific to each purpose, so reminders and marketing need separate opt-ins.

10.3 An AI-use policy for your clinic

Use it when: The ten-minute document that answers the question a regulator will ask.

COPY-PASTE PROMPT

You are a healthcare compliance writer in India. Draft a one-page AI-use policy for a [specialty] clinic covering: permitted uses (drafting non-clinical communication and patient education); prohibited uses (no patient identifiers, no clinical decisions, no direct patient counselling or prescribing); the requirement that a doctor reviews every output; data handling under the DPDP Act 2023; where records are stored; and who owns the policy. Use numbered clauses, under 500 words.

Watch out: Date it, name an owner, and review it every six months. A policy nobody owns is a policy nobody follows.

10.4 A de-identify-this-data prompt

Use it when: The habit that makes every other prompt safe.

COPY-PASTE PROMPT

You are a data-privacy assistant. I will paste a piece of patient-related text. Replace every direct and indirect identifier with a placeholder: names become [PATIENT], phone numbers [PHONE], addresses [ADDRESS], dates of birth [DOB], ABHA or hospital numbers [ID], and any rare detail that could identify the person becomes [DETAIL]. Keep all clinical information unchanged. Return only the de-identified text and a list of what you replaced. Here is the text: [paste].

Watch out: Run this before any other prompt that needs patient context. Then check the output yourself - de-identification is never perfect.

10.5 A wording check for any marketing text

Use it when: Run every clinic post through this filter before it goes live.

COPY-PASTE PROMPT

You are a healthcare compliance reviewer in India. Review the following clinic marketing text against the Indian Medical Council (Professional Conduct, Etiquette and Ethics) Regulations, 2002. Flag any phrase that solicits patients, invites attention to a doctor's skill or achievements, uses superlatives, promises outcomes, offers incentives, or reveals patient information. For each flagged phrase, suggest a compliant replacement. Present as a table: phrase, problem, replacement. Here is the text: [paste].

Watch out: Run this on every post, ad and profile description. It takes seconds and catches the mistakes that cost registrations.

10.6 A DPDP breach-notification draft

Use it when: If the worst happens, you should not be writing this from scratch.

COPY-PASTE PROMPT

You are a data-protection writer in India. Draft a template notification for a personal data breach at a [specialty] clinic, addressed to affected patients and to the Data Protection Board of India under the Digital Personal Data Protection Act, 2023. Include: what happened in plain words; what data was involved; what the clinic has done; what the patient should do; and whom to contact. Provide two versions - one short patient message, one formal letter - with placeholders for the specifics.

Watch out: Under the DPDP Act, a breach must be reported to the Board and to each affected patient. Know who does it before you need to.

11. Regional Languages & Accessibility

Most Indian patients are more comfortable in a language other than English. These prompts make your patient material genuinely usable - in the right language, at the right reading level.

11.1 A Hindi patient-education leaflet

Use it when: Translate and localise, not just translate.

COPY-PASTE PROMPT

You are a health-communication writer in India. Take the following patient-education text and produce a Hindi version written in Devanagari, using everyday Hindi that a patient in [state] would actually understand, not formal or Sanskritised Hindi. Keep all clinical meaning exactly. Add a line at the end telling the patient to consult their doctor. Here is the text: [paste English text].

Watch out: Ask for a plain-Hindi version, not a literary one. Everyday Hindi is what gets read.

11.2 Translate patient material into another Indian language

Use it when: Tamil, Marathi, Bengali, Telugu, Kannada, Gujarati - the same method each time.

COPY-PASTE PROMPT

You are a health-communication translator in India. Translate the following patient-education text into [language], in that language's own script, using everyday spoken register rather than formal literary style. Preserve all clinical meaning exactly, keep medicine names as I have written them, and do not add or remove any advice. Add a closing line telling the patient to consult their doctor. Here is the text: [paste].

Watch out: Have a native speaker on your staff read the final version before you print it. Machine translation misses nuance.

11.3 Rewrite for low literacy

Use it when: Plain language is not the same as simple language.

COPY-PASTE PROMPT

You are a health-literacy specialist in India. Rewrite the following patient-education text so that a person with limited reading ability can understand it: use short sentences; common everyday words; one idea per line; and where a concept is abstract, give a concrete everyday example. Do not remove any clinical information, especially not warning signs. Keep it under [word count]. Here is the text: [paste].

Watch out: Danger signs must survive every rewrite. Check them first after the AI is done.

11.4 An IVR or phone greeting script

Use it when: Your clinic's phone system should speak the patient's language.

COPY-PASTE PROMPT

You are a clinic operations writer in India. Write a short IVR or phone-greeting script for a [specialty] clinic, providing versions in English, Hindi and [another language]. It should cover: the clinic's name and timings; the option to book an appointment; the option for an emergency; and a line directing the caller to the website. Each version under 60 words, warm and clear.

Watch out: Offer the emergency option first in the menu. In healthcare, that order matters.

11.5 A multilingual WhatsApp auto-reply set

Use it when: Meet the patient in the language they wrote in.

COPY-PASTE PROMPT

You are a clinic operations writer in India. Write a set of five WhatsApp auto-reply messages for a [specialty] clinic - greeting, appointment request received, out-of-hours, directions, and a request to send reports - each in English and in [language]. Each message under 45 words, polite, factual, and free of any medical advice. Add one line telling the patient how to reach a person.

Watch out: Auto-replies must never give medical advice. Keep them purely administrative.

The 30-Day Rollout

Do not try to use all 85 prompts in a week. Pick the workflow below, spend fifteen minutes a day, and by day thirty the clinic will be running on a different operating rhythm.

Week	Focus	Prompts to run
Week 1	Patient communication	Section 1 - run all 12. Save the ones you will reuse as templates on your phone.
Week 2	Getting found	Section 4 (Google Business Profile) - build the profile, add the services list and the Q&A.
Week 3	Reputation and content	Section 5 (Reviews) and Section 6 (Content) - set up the review flow and the 30-day calendar.
Week 4	Admin and compliance	Section 7 (OPD admin), Section 10 (Compliance) - draft the AI-use policy and the consent notice.

The daily fifteen minutes

1. Morning: run one Section 1 prompt to clear the overnight WhatsApp backlog.
2. Midday: run one Section 6 prompt and post it.
3. Evening: reply to every new review using the Section 5 templates.
4. Once a week: run the Section 4 ten-minute profile audit.
5. Once a month: run the Section 9 practice-review prompt and the Section 5 reputation report.

How to keep the quality high

- Keep a running document of the prompts that worked for your specialty, with your own edits saved in.
- Review any AI output that goes to a patient the same way you would review a junior colleague's draft.
- Re-read the Compliance Primer every six months, and after any change to the DPDP Rules or the NMC regulations.
- If a prompt produces something wrong, fix the prompt rather than the output - the fix will save you time every time after.

Quick Reference Card

Print this page and pin it near the front desk or your desk.

The three rules

1. No patient identifiers in any AI tool - ever.
2. The doctor, not the AI, makes every clinical decision.
3. Every AI output is a draft you must review and own.

The five-part prompt formula

Role - Task - Context - Format - Guardrail. Drop the guardrail and you get confident nonsense.

Never type or publish

Never	Because
A patient's name, phone, address, ABHA or hospital number, photo or DOB	DPDP Act 2023 - health data is sensitive personal data
"Best", "most trusted", "guaranteed", "cure"	NMC Regulations 6.1 and 7.12 - no advertising that invites attention to your skill
A public reply that confirms someone was a patient	NMC Regulation 7.14 and the DPDP Act - a fresh disclosure
An AI-drafted prescription or direct patient counselling	NMC Telemedicine Practice Guidelines, 2020 - only a registered practitioner may do this
A free check-up or discount in exchange for a review	Google and Practo policies - can cost you the profile

Numbers worth remembering

Figure	Source
13,88,185 registered allopathic doctors in India	Press Information Bureau, February 2026
Doctor-to-population ratio of about 1:811	Press Information Bureau, February 2026
About 68% of first-time metro patient bookings start on Google Maps	Ichelon Consulting, 2026
Average Google rating 4.78 stars; about 22.9 new reviews a month	Study of 245 Indian healthcare brands, October 2026
Only about 60.5% of reviews get a reply	Same study, October 2026
Staff and bedside manner drive 23% of negative reviews	Same study, October 2026
Wait time and scheduling drive 18.8% of negative reviews	Same study, October 2026

If you remember only one thing

A message sent under your clinic's name is your professional statement, no matter who typed the first draft. Use AI to write faster. Keep the judgement, the consent and the signature for yourself.

Sources & Further Reading

Every factual claim in this pack traces to one of the official or published sources below. Where a figure is from a survey rather than an official count, it is described as such in the text.

Source	What it covers
NMC / MCI Telemedicine Practice Guidelines, 2020 (with NITI Aayog) - Appendix 5 to the IMC (Professional Conduct, Etiquette and Ethics) Regulations, 2002	Consent, identity, record-keeping, prescribing, and the rule that AI platforms may not counsel or prescribe
Indian Medical Council (Professional Conduct, Etiquette and Ethics) Regulations, 2002 (amended to 2016) - nmc.org.in	Advertising and soliciting (6.1), self-photographs (6.1.2), lay press (7.11, 7.12), confidentiality (7.14), records (1.3), display of registration number (1.4.1), generic prescribing (1.5)
The Digital Personal Data Protection Act, 2023, and the DPDP Rules, 2025	Consent, notice, children's data, breach notification, patient rights, penalties
Ayushman Bharat Digital Mission, National Health Authority - abdm.gov.in	ABHA, Healthcare Professionals Registry, Health Facility Registry, consent-based record exchange
Press Information Bureau, Government of India, February 2026	Registered doctors and AYUSH practitioners, doctor-to-population ratio, nursing workforce
Google Business Profile help and user-contributed content policies	Profile rules, review policies, what may and may not be done with reviews
Practo provider guidelines	Healthcare review and reply rules on the Practo platform
Ichelon Consulting, "State of India Healthcare Marketing 2026" (245 brands, October 2026) and 2026 healthcare-marketing analyses	Google review statistics, booking behaviour, reputation-management practice

This pack is an educational and productivity resource. It is not medical, legal or regulatory advice, and it does not replace the NMC Telemedicine Practice Guidelines, the NMC Professional Conduct regulations, the DPDP Act 2023, or the judgement of a registered medical practitioner. Always confirm current requirements against the official source before acting.